



Division of Provider Services & Quality Assurance
Office of Long-Term Care
P.O. Box 8059, Slot S404, Little Rock, AR 72203-1437

Notice of Serious Incident

Case Number: 032012

Date of Incident: 08/10/2025

Date Received: 08/11/2025

Facility Name: United Methodist Children's Home Little Rock Campus

Incident Type: DPSQA - Incident & Accident - OLTC

Case Status: Under Review

DPSQA - Incident & Accident - OLTC

Report Description: At approximately 5:45 p.m., staff member [REDACTED] entered the cafeteria and engaged in a verbal altercation with a peer. Staff instructed [REDACTED] to calm down and relax; however, he refused and continued the verbal exchange. [REDACTED] (staff) asked [REDACTED] to step into the hallway to de-escalate, but [REDACTED] declined and questioned the request. [REDACTED] (staff) attempted to address [REDACTED] negative behaviors using the Teaching Family Model. [REDACTED] then stood up, exited to the hallway, and forcefully slammed the door. Staff allowed him a few minutes to calm down before attempting to re-engage. Upon re-entering the cafeteria, [REDACTED] continued to display agitation. When approached by [REDACTED] she determined he was still not calm enough to rejoin the group and informed [REDACTED]. [REDACTED] began using profanity toward staff, verbally refusing directives, and making statements about what he would not do. [REDACTED] attempted to speak with [REDACTED] again, but [REDACTED] continued cursing and calling staff derogatory names. [REDACTED] then pushed [REDACTED]. [REDACTED] and [REDACTED] attempted to place him in a two-person restraint; however, [REDACTED] was unable to maintain hold, causing [REDACTED] and [REDACTED] to make contact with the wall during the restraint attempt. [REDACTED] was released from the restraint and exited the cafeteria, forcefully kicking open the classroom doors. He continued yelling and subsequently activated the fire alarm.

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NAME OF PROVIDER OR SUPPLIER UNITED METHODIST CHILDRENS HOME		STREET ADDRESS, CITY, STATE, ZIP CODE 2002 S FILLMORE ST LITTLE ROCK, AR 72214			
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- 000	Initial Comment The facility was not in compliance with Title 20, Chapter 10. Office of Long Term Care, Division of Provider Services and Quality Assurance, Department of Human Services Part 415. Psychiatric Residential Treatment Facility Providers. ELS 032012	- 000			
-501 SS=D	Use of restraints (a)(1) The United States Children's Health Act of 2000 (Public Law No. 106-310) imposes procedural reporting and training requirements regarding the use of restraints and involuntary seclusion in Psychiatric Residential Treatment Facilities that provide inpatient psychiatric services for children who are under twenty-one (21) years of age. (2) Satellite training by the Centers for Medicare & Medicaid Services may be accessed on the Centers for Medicare & Medicaid Services website at www.cms.gov . Federal regulations are located at 42 C.F.R. § 483, Subpart G (2001). (3) The use of mechanical restraints and seclusions are strictly prohibited. (b) A Psychiatric Residential Treatment Facility will report monthly on an approved Office of Long-Term Care template, to the State-designated Protection and Advocacy system, the Office of Long-Term Care, and the Resident's PASSE, or the Resident's sending state the following data: (1) Physical restraints used within the Psychiatric Residential Treatment Facility, in the aggregate and by client; (2) Chemical restraints used within the Psychiatric Residential Treatment Facility, in the aggregate and by client;	-501			

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

Nicole H. Full, LMSW

STATE FORM

TITLE

Program Director

D7UN11

(X6) DATE

6899

If continuation sheet 1 of 16

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-501	Continued From page 1 (3) Sedating PRNs within the Psychiatric Residential Treatment Facility, in the aggregate and by client; and (4) Ratio of chemical restraints to sedating PRNs, in the aggregate and by client. (c) An order for restraint must not be written as a standing order or on an as-needed basis. (d) Restraint must not result in harm or injury to the Resident and must be used only: (1) To ensure the safety of the Resident or others during an emergency safety situation; and (2) Until the emergency safety situation has ceased, and the Resident's safety and the safety of others can be ensured, even if the restraint order has not expired. (e) Restraint must not be used simultaneously. (f) Orders for restraint must be ordered by a physician, or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint or seclusion and trained in the use of emergency safety interventions. (g) If the Resident's treatment team physician is available, only he or she can order a restraint. (h) A physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order a restraint must order the least restrictive emergency safety intervention that is most likely to be effective in resolving the emergency safety situation based on consultation with staff. (i)(1) If the order for restraint is verbal, the verbal order must be received by a registered nurse or other licensed staff such as a licensed practical nurse, while the emergency safety intervention is being initiated by staff or immediately after the emergency safety situation	-501		

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-501	Continued From page 2 ends. (2) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must verify the verbal order in a signed written form in the Resident's record. (3) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must be available to staff for consultation, at least by telephone, throughout the period of the emergency safety intervention. (j) Each order for restraint must: (1) Be limited to no longer than the duration of the emergency safety situation; and (2) Under no circumstances exceed four (4) hours for Residents who are between eighteen (18) years of age and twenty-one (21) years of age; two (2) hours for Residents who are between nine (9) years of age and seventeen (17) years of age; or one (1) hour for Residents who are under nine (9) years of age. (k) Within one (1) hour of the initiation of the emergency safety intervention a physician, or other licensed practitioner trained in the use of emergency safety interventions and permitted by the state and the Psychiatric Residential Treatment Facility to assess the physical and psychological well-being of Residents, must conduct a face-to-face assessment of the physical and psychological well-being of the Resident, including but not limited to: (1) The Resident's physical and psychological status; (2) The Resident's behavior; (3) The appropriateness of the intervention measures; and (4) Any complications resulting from the intervention.	-501		

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-501	Continued From page 3 (l) Emergency safety intervention: An emergency safety intervention must be performed in a manner that is safe and proportionate and that is appropriate to the severity of the behavior and to the Resident's chronological and developmental age; size; gender; physical, medical, and psychiatric condition; and personal history (including any history of physical or sexual abuse). (m) Staff must document the intervention in the Resident's record. That documentation must be completed by the end of the shift in which the intervention occurs. If the intervention does not end during the shift in which it began, documentation must be completed during the shift in which it ends. Documentation must include all of the following: (1) Each order for restraint; (2) The time the emergency safety intervention actually began and ended; (3) The time and results of the one (1) hour physical and psychological assessment required via face-to-face of the well-being of the Resident as required above; (4) The emergency safety situation that required the Resident to be restrained; and (5) The name of staff involved in the emergency safety intervention. (n) The Psychiatric Residential Treatment Facility must maintain a record of each emergency safety situation, the interventions used, and their outcomes. (o) The physician or other licensed practitioner permitted by the state and the Psychiatric Residential Treatment Facility to order restraint must sign the restraint or seclusion order in the Resident's record as soon as possible. Authority. Arkansas Code §§ 20-76-201 et seq.,	-501			

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-501	Continued From page 4 and 25-10-129. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on observations, interviews, and record and video review, the facility failed to prevent injury during a restraint and failed to document the required information for a restraint for one (Client #1) of three clients reviewed for injuries. The findings include: Client #1 Review of Client #1's "Master Treatment Plan" dated 05/12/2025, revealed Client #1 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, unspecified disruptive impulse control, conduct disorder, attention deficient hyperactivity disorder, child neglect or abandonment confirmed, child physical abuse confirmed, and borderline intellectual functioning. Review of the "Restraint and Seclusion" packet dated 08/09/25 and completed by Registered Nurse (RN) #4, revealed that Client #1 was placed in a physical restraint at 5:45 PM and was released at 5:45 PM. The Behavioral Instructor (BI) #2 documented Client #1 pushed staff then BI #2 and BI #1 attempted to place Client #1 in a two-man team hold. The Restraint and Seclusion documentation noted the interventions attempted before restraints were implemented as time out and verbal de-escalation. Review of the "Client Debrief" within the	-501	-501 The staff member directly involved in the improper restraint was immediately suspended and later terminated following video review. The two other staff members present during the restraint received written disciplinary action and re-education. In-service trainings were held for all Behavioral Instructors on 8/28/25 (2nd shift) and 8/29/25 (1st shift), with Weekend Option staff receiving the training via email and required to acknowledge on 9/9/25. The trainings included: Phase 2 tantrum management (managing escalating behaviors) Proper restraint requirements and documentation Consultant notification procedures Ongoing review and discussion of CPI procedures, restraint implementation, and tantrum management will be incorporated into monthly Consultation sessions by Program Consultants, with documentation reflecting identified areas of need and follow-up review with Behavioral Instructors.	9/09/2025

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-501	Continued From page 5 "Restraint and Seclusion" packet indicated Client #1 stated 'staff got in my personal space, and I got mad and pushed him in the face'. Review of "Nursing Progress Note" dated 08/09/2025 at 5:40 PM included details of a team restraint being attempted without success at 5:45 PM. At 6:10 PM, RN #4 documented Client #1 was assessed by the nurse. A small laceration to right index finger, abrasion to right side of neck, swelling to the right eye with some discoloration was noted. Review of the Face-to-Face Assessment within the "Restraint and Seclusion" packet completed by RN #4 on 08/09/25 at 6:10 PM does not identify any injuries. Review of the video footage dated 08/09/25, time stamp beginning at 5:33:38 PM and ending at 5:47:54 PM, was reviewed with the Director of Residential Treatment Center (RTC) and Consultant #2, revealed at 5:34:10 PM BI #2 stated "That's 30." to Client #1 and gestured toward the exit of the room. Client #1 exited the cafeteria and slammed the cafeteria door. At 5:34:29 BI #2 was heard stating to Client #1 "I'm trying to tell you; I'm going to teach to you." At 5:42:36 PM Client #1 reentered the cafeteria through the southeastern entrance. At 5:42:47 PM BI #1 stated, "Client #1 can't eat yet. Not until they hear what I have to say." At 5:42:55 PM BI #2 stated to Client #1 "You can go back into the hallway." A verbal exchange was heard between BI #1, BI#2 and Client #2. At 5:43:14 PM BI #2 stated to Client #1 "Tomorrow you'll be stuck and won't do nothing. Keep talking, you're not tough, stop trying. Mind your business." At 5:44:36 PM Client #1 hit the hand sanitizer,	-501			

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-501	Continued From page 6 with their right fist, on the northeastern wall of the serving line, causing a laceration on the right index finger then client #1 was observed hitting the clients head against the wall three times with increasing intensity. At 5:44:49 PM- BI #2 stated, "You're not gonna be in here doin that." BI #2 stood approximately 1 foot in front of Client #1. After a verbal exchange, Client #1 was observed pushing BI #2. At 5:45:04 PM BI #2 placed his right hand to Client #1 left shoulder near the collar. Client #1 grabbed BI #2 with their left hand to their right shoulder near their collar. BI #2 Replaced the hold, which caused Client #1's head to strike the wall. The total time for the restraint was .04 seconds. At 5:45:23 PM BI #2 placed Client #1 in a "Rear Arm Lock." (used their left and right hand to control Client #1's left arm and placing it behind their back). BI #2 used their left hand to hold Client #1's left shoulder, and their right hand to control and hold Client #1's left wrist behind their back. Client #1 turned away from the wall of the serving line and with BI #2 behind the client. During the event Client #1's hit the wall of the serving line causing the client's face to strike the wall. BI #2 used their body weight and the walls to restrain Client #1 by in the corner of the wall at the north end of the serving line. BI#2 was observed pressing their body to Client #1's body from behind. BI #2 was observed standing approximately 3 feet behind BI #2. The restraint was released at 5:45:49 PM At 5:46:08 PM Client #1 remained near the corner of the serving line. Client #1 was observed checking the clients head and face for injuries several times. During a telephone interview on 08/13/25 at 3:44 PM, BI #2 confirmed they were present and participated in an incident with Client #1 on 08/09/25. BI #2 reported Client #1 was acting out	-501		

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-501	Continued From page 7 in the cafeteria and was defiant, BI #2 and BI #1 attempted to put Client #1 in a restraint. BI #2 reported that Client #1 said one of the staff hit Client #1's head and reported Client #1 had a scratch. BI #2 declined to provide any additional information about the restraint or training. During an interview on 08/13/25 at 5:20 PM, BI #1 confirmed they were present and participated in an incident with Client #1 on 08/09/25. BI #1 revealed Client #1 was frustrated and was offered a timeout. During the time out Client #1 hit the wall and pushed BI #2. BI #1 stated after Client #1 pushed BI #2, both BI #1 and BI#2 attempted a two man hold but were unsuccessful. The interviewer inquired about any injuries sustained during the incident and BI #1 reported Client #1 said BI #2 injured them. BI #1 was unable to verify that the injury came from BI #2 because "it all happened so fast as they were going into the corner". BI#1 further stated it looked like Client #1 hit the wall. The interviewer inquired about training and training techniques for restraints and what to do if a restraint fails. BI #1 reported training in Crisis Prevention Intervention and that they are to do a two man hold, meaning one staff on each side of the client's body, and if a restraint fails, they are trained to back out of it and try again. During a telephone interview on 08/14/25 at 10:25 AM with, Registered Nurse (RN) #4, revealed BI #2 brought Client #1 to the RN after the restraint on 8/9/25. RN #4 revealed Client #1 was upset and reported the client injured their finger. RN #4 noted a scratch to the right-side of Client 1's neck and an injury to the client's right eye. RN #4 stated the physician was notified of the injury. RN #4 reported BI #1 reported Client #1 was throwing a tantrum and a restraint was attempted.	-501			

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-501	Continued From page 8 During an interview on 08/13/25 at 2:35 PM with Client #1 revealed the client was in a restraint conducted by BI #2. Client #1 presented with two scratches on the right side of their neck, one about an inch long and the other approximately one-half inch long and bruising to the right eyelid. The interviewer inquired about how Client #1 obtained the injuries, and Client #1 reported they occurred during a restraint when BI #2 "slammed their face into the wall" and scratched their neck with what they thought were keys. The interviewer inquired where the restraint occurred and Client #1 reported it occurred in the serving line in the cafeteria, when BI #2 "shoved their face in the corner". Client #1 stated they did go to the nurse after the restraint, and the nurse applied ointment on their neck. During an interview on 08/14/25 at 8:45 AM with the Director of RTC, the video footage of the injury during restraint was viewed and details of the footage were verified. Prior to the restraint the Director of RTC identified Client #1 had already received two prompts with the opportunity to follow instructions, and Client #1 was still not following instructions. At 5:43:14 PM of the video footage the Director of RTC identifies, the event was a classic example of a verbal confrontation model that was addressed in training. The Director of the RTC stated the exchange was a power struggle. The Director of RTC identified there should have been a "Swap-Out" between BI #1 and BI #2, but that did not occur. When Client #1 was observed, via video, banging their head against the wall, the interviewer inquired with the Director of RTC about the protocol for preventing self-injurious behavior. The Director of RTC explained that if a client continues self-injurious behavior or does not respond to prompts, staff	-501			

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-501	Continued From page 9 intervene to prevent the actions that could lead to injury. Proper procedures in positioning for the prevention of self injurious behaviors were inquired about and the Director of RTC revealed in Crisis Prevention Intervention (CPI)training they teach the supportive stance, which should have been used, however, BI #2 was posturing by standing in front of Client #1. The Director of RTC verified that is not in the training received by the facility. The Director of RTC confirmed the injury most likely was caused by the restraint. During an interview, concurrent with viewing the restraint video on 08/14/25 at 8:45 AM with Consultant #2, who agreed a restraint was conducted when BI #2 was using their body weight to hold Client #1 against the wall and movement was restricted. Review of the facility policy titled, "RTC Seclusion & Restraint" revised April 2024, stated 'Seclusion and/or restraint are used only with adequate justification, documentation, and regard for resident safety. All residents have the right to be free from harm, including the unnecessary or excessive use of restraint, or seclusion. Client #2 Review of Client #2's "Master Treatment Plan" dated 07/15/2025, revealed Client #2 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, post-traumatic stress disorder, attention deficient hyperactivity disorder, cannabis use, nicotine dependence, child neglect or abandonment confirmed, and child physical abuse suspected. Review of the "Restraint and Seclusion" packet dated 07/18/25, revealed Client #2 was placed in a transport (an assist to safely move an	-501			

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-501	Continued From page 10 individual from one area to another) at 2:12 PM and was released at 2:13 PM. The concise description of incident documented by Behavioral Instructor (BI) #4 revealed Client #2 was verbally aggressive before throwing items at peers and staff. Client #2 was escorted to the unit by BI #4. Client #2 continued to be verbally aggressive. Registered Nurse #3 (RN#3) attempted to provide medication, but Client #2 threw the medicine cup at the nurse. The Restraint and Seclusion documentation included time out and verbal de-escalation was attempted prior to the restraint. The "Restraint and Seclusion" packet does not identify that a chemical restraint was administered. Review of the Medication Administration Record for July 2025 revealed on 07/18/25 at 2:15 PM, Registered Nurse (RN) #3 gave Client #2 Zyprexa (an antipsychotic medication) 5 mg by mouth for increased agitation with Benadryl 50 mg by mouth for side effects. At 2:40 PM, RN #3 documented the medications given were effective. Review of "Medication Order" dated 07/18/25 at 2:15 PM, revealed the treating physician wrote a one-time order for Zyprexa 5mg by mouth for agitation and Benadryl 50 mg by mouth for agitation. That order was signed by RN #3 on 07/18/25 at 2:29 PM and the physician on 07/18/25 at 5:41 PM. An interview was conducted on 08/14/25 at 10:13 PM, with RN #3 who revealed the medication was not documented as a chemical restraint since it was given by mouth and used for agitation. RN #3 noted Client #2 was placed on unit restrictions after the medication was administered and the client rested for the remainder of the time RN #3	-501	-501 On 8/14/25, the Nurse Manager sent a memo to all nursing staff clarifying: "Until further notice, if a client receives a medication that is not listed as a PRN on their MAR, it is to be considered a chemical restraint and an S&R must be completed, even if the reason for the medication is agitation or psychosis." The incident was reviewed with nursing staff during shift huddles, and expectations for restraint documentation were reinforced. Systemic Changes Implemented: Policy clarification issued regarding chemical restraint documentation requirements. All non-PRN medications administered for agitation or psychosis must be documented as chemical restraints with a corresponding S&R form. Ongoing Monitoring: Nurse Manager will review all MARs weekly to confirm correct classification and documentation of chemical restraints.	8/18/2025

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-501	Continued From page 11 was with Client #2. RN #3 reported client #2's guardian was contacted because Client #2 was giving them difficulty taking the medication. During a phone interview on 08/13/25 at 2:48 PM, with the Nurse Manager who revealed sometimes the physician gives a one-time order for medication that would not be considered a chemical restraint. The Nurse Manager stated if the medication is not prescribed for aggression, then it is not considered a chemical restraint. Review of the facility policy titled, "RTC Seclusion & Restraint" revised April 2024, stated 'Drug Used as a Restraint (chemical restraint) means any drug that: 1. Is administered to manage a resident's behavior in a way that reduces the safety risk to the resident or others; 2. Has a temporary effect of restricting the resident's freedom of movement; and 3. Is not a standard for the resident's medical or psychiatric condition.'	-501		
-505 SS=D	Post-intervention debriefings (a)(1) Within twenty-four (24) hours after the use of restraint, staff involved in an emergency safety intervention and the Resident must have a face-to-face discussion. (2) This discussion must include all staff involved in the intervention except when the presence of a particular staff person may jeopardize the well-being of the Resident. (3) Other staff and the Resident's parent or legal guardian may participate in the discussion when it is deemed appropriate by the Psychiatric Residential Treatment Facility. (4) The Psychiatric Residential Treatment Facility must conduct such discussion in a	-505		

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NAME OF PROVIDER OR SUPPLIER UNITED METHODIST CHILDRENS HOME		STREET ADDRESS, CITY, STATE, ZIP CODE 2002 S FILLMORE ST LITTLE ROCK, AR 72214		
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-505	Continued From page 12 language that is understood by the Resident's parent or legal guardian. (5) The discussion must provide both the Resident and staff the opportunity to discuss the circumstances resulting in the use of restraint and strategies to be used by the staff, the Resident, or others that could prevent the future use of restraint or seclusion. (b) Within twenty-four (24) hours after the use of restraint, all staff involved in the emergency safety intervention, and appropriate supervisory and administrative staff, must conduct a debriefing session that includes, at a minimum, a review and discussion of: (1) The emergency safety situation that required the intervention, including a discussion of the precipitating factors that led up to the intervention; (2) Alternative techniques that might have prevented the use of the restraint; (3) The procedures, if any, that staff are to implement to prevent any recurrence of the use of restraint; and (4) The outcome of the intervention, including any injuries that may have resulted from the use of restraint. (c) The staff must document in the Resident's record that both debriefing sessions took place and must include in that documentation the names of staff who were present for the debriefing, the names of staff that were excused from the debriefing, and any changes to the Resident's treatment plan that result from the debriefings. Authority. Arkansas Code §§ 20-76-201 et seq., and 25-10-129. This ELEMENT is not met as evidenced by: Based on observations, interviews, and record	-505	-505 Based on observations, interviews, and record review, the facility failed to complete client debriefings within 24 hours and staff debriefings within 24 hours for two of three clients reviewed. One missed debriefing resulted from the restraint not being categorized correctly when oral medication was administered as a chemical restraint. A CIR tray system was placed in Behavioral Instructor (BI) offices on each unit. All incomplete CIRs, including pending debriefings, are now placed in this tray. Consultants check the tray every awake shift to ensure CIR completion and debriefings are conducted within the 24-hour requirement. An in-service training with Program Consultants was conducted on 9/11/25 to reinforce the requirement for client and staff debriefings to be completed within 24 hours of a restraint.	9/11/2025

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-505	Continued From page 13 review, the facility failed to debrief the client within 24 hours after a restraint was implemented and failed to debrief staff within 24 hours after the use of a restraint for two (Client #2 and #3) of three clients reviewed. The findings include: Client #2 Review of Client #2's "Master Treatment Plan" dated 07/15/2025, revealed Client #2 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, post-traumatic stress disorder, attention deficient hyperactivity disorder, cannabis use, nicotine dependence, child neglect or abandonment confirmed, and child physical abuse suspected. Review of the "Restraint and Seclusion" packet dated 07/18/25, revealed Client #2 was placed in a transport (an assist to safely move an individual from one area to another) at 2:12 PM and was released at 2:13 PM. The concise description of incident documented by Behavioral Instructor (BI) #4 revealed Client #2 was verbally aggressive before throwing items at peers and staff. Client #2 was escorted to the unit by BI #4. Client #2 continued to be verbally aggressive. Registered Nurse #3 (RN#3) attempted to provide medication, but Client #2 threw the medicine cup at the nurse. The Restraint and Seclusion documentation included time out and verbal de-escalation was attempted prior to the restraint. The "Restraint and Seclusion" packet does not identify that a chemical restraint was administered. The "Restraint and Seclusion" packet revealed the staff debriefing occurred 07/21/25 at 4:16 PM.	-505		

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-505	Continued From page 14 Review of the "Restraint and Seclusion" packet dated 07/26/25, revealed Client #2 was placed in a basket hold, a physical restraint that involves wrapping an arm around the upper or lower body to prevent harm to self or others, at 10:18 AM and was released at 10:39 AM. The concise description of the incident noted Client #2 was physically aggressive with staff. The Restraint and Seclusion documentation included interventions attempted before restraints were implemented as verbal de-escalation. The "Restraint and Seclusion" packet revealed the client debriefing occurred 07/27/25 at 12:15 PM. Client #3 Review of Client #3's "Master Treatment Plan" dated 05/30/2025, revealed Client #3 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, oppositional defiant disorder, attention deficient hyperactivity disorder, and child physical abuse suspected. Review of the "Restraint and Seclusion" packet dated 07/25/25, revealed Client #3 was placed in a transport, (an assist to safely move an individual from one area to another) at 8:45 AM and was released at 8:45 AM. The concise description of incident noted Client #3 was verbally aggressive with peers and was kicking doors, punching windows and pulling aggressively at doorknobs. As aggressive behavior continued Client #3 was transported from the day room. The Restraint and Seclusion documentation noted interventions attempted before restraints were implemented as time-out and verbal de-escalation. The "Restraint and Seclusion" packet revealed the client debriefing occurred on 08/13/25 at 12:21 PM. The "Restraint and Seclusion" packet reveals that the staff	-505			

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-505	Continued From page 15 debriefing occurred 08/13/25 at 12:24 PM. During an interview 08/13/25 at 3:05 PM with consultant #1, revealed client debriefing is to occur 24 hours after the initiation of any restraint. During a phone interview 08/13/25 at 2:48 PM the Nurse Manager, revealed client debriefing is to occur 24 hours after the initiation of any restraint. Review of the facility policy titled, "RTC Seclusion & Restraint" revised April 2024, stated 'Debriefing (debriefing with staff & resident) 1. As soon as possible, but at least within 24 hours after the use of the restraint or seclusion, staff involved in an emergency safety intervention and the resident must have a face-to-face discussion.'	-505			

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N 128	PROTECTION OF RESIDENTS CFR(s): 483.356(a)(3) Restraint or seclusion must not result in harm or injury to the resident and must be used only- This ELEMENT is not met as evidenced by: Based on observations, interviews, and record and video review, the facility failed to prevent injury during a restraint and failed to document the required information for a restraint for one (Client #1) of three clients reviewed for injuries. The findings include: Client #1 Review of Client #1's "Master Treatment Plan" dated 05/12/2025, revealed Client #1 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, unspecified disruptive impulse control, conduct disorder, attention deficient hyperactivity disorder, child neglect or abandonment confirmed, child physical abuse confirmed, and borderline intellectual functioning. Review of the "Restraint and Seclusion" packet dated 08/09/25 and completed by Registered Nurse (RN) #4, revealed that Client #1 was placed in a physical restraint at 5:45 PM and was released at 5:45 PM. The Behavioral Instructor (BI) #2 documented Client #1 pushed staff then BI #2 and BI #1 attempted to place Client #1 in a two-man team hold. The Restraint and Seclusion documentation noted the interventions attempted before restraints were implemented as time out and verbal de-escalation. Review of the "Client Debrief" within the	N 128	N 128 The staff member directly involved in the improper restraint was immediately suspended and later terminated following video review. The two other staff members present during the restraint received written disciplinary action and re-education. In-service trainings were held for all Behavioral Instructors on 8/28/25 (2nd shift) and 8/29/25 (1st shift), with Weekend Option staff receiving the training via email and required to acknowledge on 9/9/25. The trainings included: Phase 2 tantrum management (managing escalating behaviors) Proper restraint requirements and documentation Consultant notification procedures Ongoing review and discussion of CPI procedures, restraint implementation, and tantrum management will be incorporated into monthly Consultation sessions by Program Consultants, with documentation reflecting identified areas of need and follow-up review with Behavioral Instructors.	9/09/2025	

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

Nicole Hull, LMSW

TITLE

Program Director

(X6) DATE

Any deficiency statement ending with an asterisk (*) denotes a deficiency which the institution may be excused from correcting providing it is determined that other safeguards provide sufficient protection to the patients. (See instructions.) Except for nursing homes, the findings stated above are disclosable 90 days following the date of survey whether or not a plan of correction is provided. For nursing homes, the above findings and plans of correction are disclosable 14 days following the date these documents are made available to the facility. If deficiencies are cited, an approved plan of correction is requisite to continued program participation.

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N 128	Continued From page 1 "Restraint and Seclusion" packet indicated Client #1 stated 'staff got in my personal space, and I got mad and pushed him in the face'. Review of "Nursing Progress Note" dated 08/09/2025 at 5:40 PM included details of a team restraint being attempted without success at 5:45 PM. At 6:10 PM, RN #4 documented Client #1 was assessed by the nurse. A small laceration to right index finger, abrasion to right side of neck, swelling to the right eye with some discoloration was noted. Review of the Face-to-Face Assessment within the "Restraint and Seclusion" packet completed by RN #4 on 08/09/25 at 6:10 PM does not identify any injuries. Review of the video footage dated 08/09/25, time stamp beginning at 5:33:38 PM and ending at 5:47:54 PM, was reviewed with the Director of Residential Treatment Center (RTC) and Consultant #2, revealed at 5:34:10 PM BI #2 stated "That's 30." to Client #1 and gestured toward the exit of the room. Client #1 exited the cafeteria and slammed the cafeteria door. At 5:34:29 BI #2 was heard stating to Client #1 "I'm trying to tell you; I'm going to teach to you." At 5:42:36 PM Client #1 reentered the cafeteria through the southeastern entrance. At 5:42:47 PM BI #1 stated, "Client #1 can't eat yet. Not until they hear what I have to say." At 5:42:55 PM BI #2 stated to Client #1 "You can go back into the hallway." A verbal exchange was heard between BI #1, BI#2 and Client #2. At 5:43:14 PM BI #2 stated to Client #1 "Tomorrow you'll be stuck and won't do nothing. Keep talking, you're not tough, stop trying. Mind your business."	N 128			

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N 128	Continued From page 2 At 5:44:36 PM Client #1 hit the hand sanitizer, with their right fist, on the northeastern wall of the serving line, causing a laceration on the right index finger then client #1 was observed hitting the clients head against the wall three times with increasing intensity. At 5:44:49 PM- BI #2 stated, "You're not gonna be in here doin that." BI #2 stood approximately 1 foot in front of Client #1. After a verbal exchange, Client #1 was observed pushing BI #2. At 5:45:04 PM BI #2 placed his right hand to Client #1 left shoulder near the collar. Client #1 grabbed BI #2 with their left hand to their right shoulder near their collar. BI #2 Replaced the hold, which caused Client #1's head to strike the wall. The total time for the restraint was .04 seconds. At 5:45:23 PM BI #2 placed Client #1 in a "Rear Arm Lock." (used their left and right hand to control Client #1's left arm and placing it behind their back). BI #2 used their left hand to hold Client #1's left shoulder, and their right hand to control and hold Client #1's left wrist behind their back. Client #1 turned away from the wall of the serving line and with BI #2 behind the client. During the event Client #1's hit the wall of the serving line causing the client's face to strike the wall. BI #2 used their body weight and the walls to restrain Client #1 by in the corner of the wall at the north end of the serving line. BI#2 was observed pressing their body to Client #1's body from behind. BI #2 was observed standing approximately 3 feet behind BI #2. The restraint was released at 5:45:49 PM At 5:46:08 PM Client #1 remained near the corner of the serving line. Client #1 was observed checking the clients head and face for injuries several times. During a telephone interview on 08/13/25 at 3:44 PM, BI #2 confirmed they were present and	N 128			

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N 128	Continued From page 3 participated in an incident with Client #1 on 08/09/25. BI #2 reported Client #1 was acting out in the cafeteria and was defiant, BI #2 and BI #1 attempted to put Client #1 in a restraint. BI #2 reported that Client #1 said one of the staff hit Client #1's head and reported Client #1 had a scratch. BI #2 declined to provide any additional information about the restraint or training. During an interview on 08/13/25 at 5:20 PM, BI #1 confirmed they were present and participated in an incident with Client #1 on 08/09/25. BI #1 revealed Client #1 was frustrated and was offered a timeout. During the time out Client #1 hit the wall and pushed BI #2. BI #1 stated after Client #1 pushed BI #2, both BI #1 and BI#2 attempted a two man hold but were unsuccessful. The interviewer inquired about any injuries sustained during the incident and BI #1 reported Client #1said BI #2 injured them. BI #1 was unable to verify that the injury came from BI #2 because "it all happened so fast as they were going into the corner". BI#1 further stated it looked like Client #1 hit the wall. The interviewer inquired about training and training techniques for restraints and what to do if a restraint fails. BI #1 reported training in Crisis Prevention Intervention and that they are to do a two man hold, meaning one staff on each side of the client's body, and if a restraint fails, they are trained to back out of it and try again. During a telephone interview on 08/14/25 at 10:25 AM with, Registered Nurse (RN) #4, revealed BI #2 brought Client #1 to the RNafter the restraint on 8/9/25. RN #4 revealed Client #1 was upset and reported the client injured their finger. RN #4 noted a scratch to the right-side of Client 1's neck and an injury to the client's right eye. RN #4	N 128			

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N 128	Continued From page 4 stated the physician was notified of the injury. RN #4 reported BI #1 reported Client #1 was throwing a tantrum and a restraint was attempted. During an interview on 08/13/25 at 2:35 PM with Client #1 revealed the client was in a restraint conducted by BI #2. Client #1 presented with two scratches on the right side of their neck, one about an inch long and the other approximately one-half inch long and bruising to the right eyelid. The interviewer inquired about how Client #1 obtained the injuries, and Client #1 reported they occurred during a restraint when BI #2 "slammed their face into the wall" and scratched their neck with what they thought were keys. The interviewer inquired where the restraint occurred and Client #1 reported it occurred in the serving line in the cafeteria, when BI #2 "shoved their face in the corner". Client #1 stated they did go to the nurse after the restraint, and the nurse applied ointment on their neck. During an interview on 08/14/25 at 8:45 AM with the Director of RTC, the video footage of the injury during restraint was viewed and details of the footage were verified. Prior to the restraint the Director of RTC identified Client #1 had already received two prompts with the opportunity to follow instructions, and Client #1 was still not following instructions. At 5:43:14 PM of the video footage the Director of RTC identifies, the event was a classic example of a verbal confrontation model that was addressed in training. The Director of the RTC stated the exchange was a power struggle. The Director of RTC identified there should have been a "Swap-Out" between BI #1 and BI #2, but that did not occur. When Client #1 was observed, via video, banging their head against the wall, the interviewer inquired with the	N 128		8/18/2025	

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N 128	Continued From page 5 Director of RTC about the protocol for preventing self-injurious behavior. The Director of RTC explained that if a client continues self-injurious behavior or does not respond to prompts, staff intervene to prevent the actions that could lead to injury. Proper procedures in positioning for the prevention of self injurious behaviors were inquired about and the Director of RTC revealed in Crisis Prevention Intervention (CPI)training they teach the supportive stance, which should have been used, however, BI #2 was posturing by standing in front of Client #1. The Director of RTC verified that is not in the training received by the facility. The Director of RTC confirmed the injury most likely was caused by the restraint. During an interview, concurrent with viewing the restraint video on 08/14/25 at 8:45 AM with Consultant #2, who agreed a restraint was conducted when BI #2 was using their body weight to hold Client #1 against the wall and movement was restricted. Review of the facility policy titled, "RTC Seclusion & Restraint" revised April 2024, stated 'Seclusion and/or restraint are used only with adequate justification, documentation, and regard for resident safety. All residents have the right to be free from harm, including the unnecessary or excessive use of restraint, or seclusion. Client #2 Review of Client #2's "Master Treatment Plan" dated 07/15/2025, revealed Client #2 had diagnoses which included major depressive disorder which was recurrent and severe without psychosis, post-traumatic stress disorder, attention deficient hyperactivity disorder, cannabis use, nicotine dependence, child neglect or abandonment confirmed, and child physical	N 128			

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N 128	Continued From page 6 abuse suspected. Review of the "Restraint and Seclusion" packet dated 07/18/25, revealed Client #2 was placed in a transport (an assist to safely move an individual from one area to another) at 2:12 PM and was released at 2:13 PM. The concise description of incident documented by Behavioral Instructor (BI) #4 revealed Client #2 was verbally aggressive before throwing items at peers and staff. Client #2 was escorted to the unit by BI #4. Client #2 continued to be verbally aggressive. Registered Nurse #3 (RN#3) attempted to provide medication, but Client #2 threw the medicine cup at the nurse. The Restraint and Seclusion documentation included time out and verbal de-escalation was attempted prior to the restraint. The "Restraint and Seclusion" packet does not identify that a chemical restraint was administered. Review of the Medication Administration Record for July 2025 revealed on 07/18/25 at 2:15 PM, Registered Nurse (RN) #3 gave Client #2 Zyprexa (an antipsychotic medication) 5 mg by mouth for increased agitation with Benadryl 50 mg by mouth for side effects. At 2:40 PM, RN #3 documented the medications given were effective. Review of "Medication Order" dated 07/18/25 at 2:15 PM, revealed the treating physician wrote a one-time order for Zyprexa 5mg by mouth for agitation and Benadryl 50 mg by mouth for agitation. That order was signed by RN #3 on 07/18/25 at 2:29 PM and the physician on 07/18/25 at 5:41 PM. An interview was conducted on 08/14/25 at 10:13	N 128			

DEPARTMENT OF HEALTH AND HUMAN SERVICES
CENTERS FOR MEDICARE & MEDICAID SERVICES

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FORM APPROVED
OMB NO. 0938-0391

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 04L106	(X2) MULTIPLE CONSTRUCTION A. BUILDING _____ B. WING _____		(X3) DATE SURVEY COMPLETED C 08/14/2025
NAME OF PROVIDER OR SUPPLIER UNITED METHODIST CHILDRENS HOME			STREET ADDRESS, CITY, STATE, ZIP CODE 2002 S FILLMORE ST LITTLE ROCK, AR 72214		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETION DATE	
N 128	Continued From page 7 PM, with RN #3 who revealed the medication was not documented as a chemical restraint since it was given by mouth and used for agitation. RN #3 noted Client #2 was placed on unit restrictions after the medication was administered and the client rested for the remainder of the time RN #3 was with Client #2. RN #3 reported client #2's guardian was contacted because Client #2 was giving them difficulty taking the medication. During a phone interview on 08/13/25 at 2:48 PM, with the Nurse Manager who revealed sometimes the physician gives a one-time order for medication that would not be considered a chemical restraint. The Nurse Manager stated if the medication is not prescribed for aggression, then it is not considered a chemical restraint. Review of the facility policy titled, "RTC Seclusion & Restraint" revised April 2024, stated 'Drug Used as a Restraint (chemical restraint) means any drug that: 1. Is administered to manage a resident's behavior in a way that reduces the safety risk to the resident or others; 2. Has a temporary effect of restricting the resident's freedom of movement; and 3. Is not a standard for the resident's medical or psychiatric condition.'	N 128	N 128 On 8/14/25, the Nurse Manager sent a memo to all nursing staff clarifying: "Until further notice, if a client receives a medication that is not listed as a PRN on their MAR, it is to be considered a chemical restraint and an S&R must be completed, even if the reason for the medication is agitation or psychosis." The incident was reviewed with nursing staff during shift huddles, and expectations for restraint documentation were reinforced. Systemic Changes Implemented: Policy clarification issued regarding chemical restraint documentation requirements. All non-PRN medications administered for agitation or psychosis must be documented as chemical restraints with a corresponding S&R form. Ongoing Monitoring: Nurse Manager will review all MARs weekly to confirm correct classification and documentation of chemical restraints.	8/18/2025	